

Stop losing reimbursable days to payer paperwork.

Medicare Advantage authorizations, continued-stay reviews, and appeals now dominate your business office — and every late or thin packet costs covered days and census. amux assembles, submits, and tracks it automatically, inside your building, on systems you already run. Nothing reaches a payer without your team's sign-off.

Runs on your own machine

Your logins, your data

HIPAA-compliant & BAA-backed

Human approves everything

WHAT WE AUTOMATE FIRST

MA authorizations & continued-stay reviews, end to end

- ✓ **Verify coverage on referral, before you accept** — Medicare, Medicaid, and MA eligibility in minutes, not hold music.
- ✓ **Assemble payer-specific auth packets** — therapy minutes, nursing notes, MDS data, and orders, pulled from your EHR.
- ✓ **Draft the clinical justification** for continued stay — your nurses and MDS coordinator review and edit.
- ✓ **Submit and follow up** through the payer or UM portal you already use — then poll for status so nobody babysits a fax.
- ✓ **Catch denials and level-of-care downgrades early** — and prep the appeal packet the same day.
- ✓ **Answer ADRs and support triple-check** — documentation matched to the claim before it goes out.

And authorizations are just the beginning. If it happens on a computer or a phone, amux can run it — referral response, census reporting, staffing schedules, family updates, month-end close. The sky is the limit.

>50%

of Medicare beneficiaries are in MA plans — every admission means auths & reviews

5–7 days

typical continued-stay review cycle per MA resident — a packet due every week

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new dashboards to learn — run it by voice or text, from your phone

Not another cloud vendor holding your PHI

amux runs on a computer **inside your building** (we can ship and manage one). It works under **your own logins**, so resident records and credentials never leave your walls. We sign a **BAA** before touching any PHI.

Your team stays in charge

amux accelerates the paperwork — it never makes a clinical or coverage decision. **Nothing clinical, coded, or billed is ever auto-submitted.** Every packet and appeal is signed off by your nurses and business office — survey-ready by design.

This is how you stay on AI's cutting edge

No AI hires, no evaluating the tool of the month. amux runs on today's **frontier AI models** and inherits every upgrade automatically — so your facility keeps compounding capability. We build your first workflow, then teach your team to build the rest — SOPs that survive turnover.

WORKS WITH THE SOFTWARE YOU ALREADY RUN

PointClickCare · MatrixCare · American HealthTech · Availity · Waystar

STEP 1

Talk to us

One call with the administrator or owner to scope your highest-leverage workflow.

STEP 2

We install in your building

On your desktop, or a Mac Studio we buy and keep online 24/7.

STEP 3

We build your first workflow

Live and running — then we teach your team to build more.

STEP 4

You run it from your phone

Watch it work, kick off tasks, approve what matters — from anywhere.

SIMPLE, TRANSPARENT PRICING

\$5,000

One-time onboarding: scope, install, and your first working workflow

\$5,000/mo

Priority support, maintenance, new feature builds, unlimited workflows

~\$200/mo

AI usage, passed through at cost — no markup, no surprises

Bring one workflow to one call.

We'll scope your highest-leverage automation live and tell you honestly whether it's a fit — before you commit to anything. Bring your administrator or business office manager.

amux.io/concierge

Compare that to one denied MA stay or a week of lost covered days — before counting business-office hours and appeal rework.

Figures reflect commonly reported industry patterns; yours will vary by payer, plan, and market. Coverage decisions are made by payers — amux drafts, assembles, submits, and tracks; your clinicians and business office review, sign, and decide. © amux